

Worcester Autism Parent Survey Findings and Recommendations

Family Needs, Public Accountability, and the Politics of Autism Support



**Survey organized by the Free Worcester Coalition in collaboration with the
Worcester Area Autism Task Force**

Report by Addison Turner

July 2026

Worcester Autism Parent Survey Findings and Recommendations

Family Needs, Public Accountability, and the Politics of Autism Support

Published by

Free Worcester Coalition

Survey leadership and development

Free Worcester Co-founder Nelly Medina

Parent Leaders Natalie Cintron and Hope Fisher

Coalition partners of Free Worcester

Research, data analysis, and report

Addison Turner

Survey period

September 2024 through January 2026

Funding

This project received partial funding through a grant awarded by the Greater Worcester Community Foundation.

Publication date

July 2026

Contact

freeworcestercoalition@gmail.com

Suggested citation

Turner, A. (2026). *Worcester Autism Parent Survey: Findings and Recommendations*. Prepared for the Free Worcester Coalition.

Permission to share

This report may be shared and reproduced for educational, advocacy, and noncommercial purposes with attribution to the Free Worcester Coalition and the report author.

Acknowledgments

The Free Worcester Coalition and the report author extend their deepest gratitude to the parents and caregivers who participated in the Worcester Autism Parent Survey. Their willingness to describe the support they have received, the barriers they continue to face, and the changes they believe are needed made this report possible.

Special recognition is due to parent leaders **Nelly Medina, Natalie Cintron, and Hope Fisher**, whose leadership helped guide the development, organization, and distribution of the survey. The survey was developed collaboratively over six months through consensus-based discussion during monthly online meetings.

We also acknowledge the coalition partners who contributed to the survey questions, supported outreach, distributed the survey through their networks, and helped ensure that the project remained connected to the experiences and priorities of Worcester families.

The survey was organized and distributed by the **Free Worcester Coalition** and its coalition partners through email, social media, and direct outreach to parents. The survey was open from September 2024 through January 2026.

This project received partial funding through a grant awarded by the **Greater Worcester Community Foundation**. That support helped make the survey process and the preparation of this report possible.

Finally, we recognize the educators, advocates, interpreters, service providers, community organizations, and family-support networks whose work contributed to outreach, discussion, and the broader local effort to improve autism support in Worcester.

The findings and recommendations presented in this report are grounded primarily in the experiences shared by participating parents and caregivers. Their voices remain the central source of evidence and the foundation for the actions proposed here.

Table of Contents

Table of Contents.....	3
1. Executive Summary.....	1
2. Introduction.....	2
2.1 Study Purpose and Premise.....	2
2.2 Autism Support and the Wider Social Infrastructure.....	2
2.3 Scope and Direction of the Report.....	3
3. Methodology.....	3
3.1 Survey Development, Distribution, and Data Preparation.....	3
3.2 Qualitative Analysis.....	5
3.3 Scope and Interpretation.....	5
3.4 Targeted Narrative Review.....	6
3.5 Local Institutional and Political Context.....	6
3.6 Study Limitations.....	6
4. Data and Results.....	6
4.1 Barriers to Getting Services and Support.....	7
4.2 School Communication, Advocacy, and IEP Navigation.....	8
4.3 Behavioral Health and Co-Occurring Service Gaps.....	10
4.4 Language Access, Trust, and Cultural Responsiveness.....	11
4.5 Parent Connection, Advocacy, and the Potential for Organization.....	12
4.6 Age and the Changing Form of Family Support Needs.....	13
5. Local Institutional and Political Context.....	15
5.1 Formal Services and Lived Access.....	15
5.2 Parent Advocacy and the 2025 Autism Resolution.....	16
5.3 The 2026 Restructuring of Specialized Programs.....	17
5.4 Implications for the Survey Findings.....	18
6. Background Research and Systemic Context.....	18
6.1 National Research and Fragmented Support Systems.....	18
6.2 Disability, Environment, and Institutional Barriers.....	19
6.3 Social Reproduction and Caregiver Burden.....	19
6.4 Promising Directions and Comparative Learning.....	20
6.5 Worcester Context and the Village as Social Infrastructure.....	20
7. Recommendations and Next Steps.....	21
Recommendations at a Glance.....	21
7.1 Immediate Service and Access Reforms.....	22
1. Establish centralized family navigation and waitlist follow-up.....	22
2. Strengthen school communication, independent IEP support, and responses to bullying.....	22
3. Guarantee meaningful language access.....	23
4. Improve behavioral health continuity and coordination.....	23

5. Address transportation, childcare, and other practical barriers.....	23
7.2 Long-Term Organization, Accountability, and System Building.....	23
1. Build and resource an autonomous parent organization.....	23
2. Establish a parent-led accountability and institutional commitment process...	24
3. Conduct comparative research and develop locally testable reforms.....	25
4. Adopt a phased public action plan.....	25
8. Conclusion.....	26
References.....	27

Acronyms and Abbreviations

Acronym	Meaning
IEP	Individualized Education Program
MOU	Memorandum of Understanding
SAIL	Specialized Approaches to Individual Learning
SPED PAC / SEPAC	Special Education Parent Advisory Council
SSI	Supplemental Security Income
WEJA	Worcester Education Justice Alliance

Report at a Glance

188 retained survey responses

175 English-language and 13 Spanish-language responses

38.8% reported being on a waiting list without adequate follow-up

79.0% reported that their child also experienced behavioral health challenges

69.5% expressed interest in becoming involved in advocacy

Five major areas of concern

- Service access and navigation
- School communication and IEP support
- Behavioral health and related services
- Language access and cultural responsiveness
- Parent connection and collective advocacy

Central conclusion

Worcester needs immediate service reforms, and parents need a longer-term process through which they can organize, hold institutions accountable, and build a more coherent social infrastructure around autistic children and their families.

How to Read This Report

This report combines structured survey results, thematic analysis of open-ended responses, relevant research, and local institutional context. The survey findings remain the primary source of evidence. The research literature and local materials are used to interpret those findings and situate them within broader patterns and current Worcester conditions.

1. Executive Summary

The report's central findings, argument, and proposed path forward.

Parents participating in this survey describe Worcester's support system for autistic children and their families as difficult to access, navigate, and rely on. Many want to stay connected and involved in advocacy, suggesting a promising foundation for grassroots organizing and systemic change.

Across the merged English- and Spanish-language survey data, four major themes came through clearly. First, **families face serious barriers to getting services and support**. Long waitlists, insurance barriers, weak coordination, unclear pathways, and practical barriers such as transportation and childcare all appear in the data. The data also demonstrate that the burden of finding, interpreting, coordinating, and sustaining support is being pushed back onto households.

Second, **schools and special education are among the few places where some families find meaningful help**. School communication, IEP follow-through, staffing, and advocacy needs remain some of the strongest pressure points. Some participating parents also described bullying, isolation, and school environments in which their children did not feel safe or fully included.

Third, **behavioral health and related support gaps emerged as one of the strongest themes in the dataset**. Behavioral health needs are widespread in this sample, but the qualitative data make clear that access is often unstable, continuity is weak, and available supports do not always match what children and families actually need.

Fourth, **language access, trust, and ongoing parent support shape whether families can use systems at all**. Interpretation barriers, lack of translated materials, cultural misunderstanding, and the need for more durable support all appear clearly in the data. Families are suffering from a lack of support they can actually understand, use, and rely on over time.

Parents are often left to supply the coordination missing among Worcester's schools, service providers, and support organizations. Worcester must strengthen these services, connect them, and give families real power over how the system is organized and held accountable.

Worcester has a formal network of autism and special education programs, and parent organizing has already compelled public recognition of autism acceptance as an important district-wide concern. Recent changes to specialized educational programs have generated serious public debate over continuity, transparency, and the role of families in decisions affecting their children. **A total of 123 respondents expressed**

interest in advocacy. That interest could strengthen the parent organizing already underway in Worcester.

Free Worcester should begin by sharing the findings with participating families and supporting parents who want to remain involved. Parent leaders and institutional representatives with decision-making authority should then develop a public plan with timelines, benchmarks, and accountability. Research into other communities can help identify approaches to coordinated care, school-family partnership, behavioral health integration, language access, and parent leadership that Worcester can adapt.

Recent program cuts attributed to long-term fiscal pressure show how the city's budgetary and administrative priorities can conflict with families' need for continuity, access, and quality. **Officials should state plainly when those pressures are driving a decision, explain how competing priorities were weighed, and involve affected families before the decision is finalized.**

2. Introduction

Why the study was undertaken and what its findings place at stake.

2.1 Study Purpose and Premise

Free Worcester is a grassroots coalition dedicated to uplifting the community of Worcester, Massachusetts, through essential services, educational programming, civic engagement, and social justice advocacy. This report grew out of a collaborative, community-led research process organized by the Free Worcester Coalition and its coalition partners, including parent leaders Nelly Medina, Natalie Cintron, and Hope Fisher.

The study formed part of a broader commitment to understanding community conditions, amplifying parent voice, and creating a basis for collective solutions in Worcester. It began from the premise that the best way to improve autism services is to listen to the parents and families attempting to access, navigate, and rely upon them. The survey was designed to identify the information needed by parents, caregivers, service providers, educational institutions, and community advocates to better understand the barriers families face and the forms of support they require.

2.2 Autism Support and the Wider Social Infrastructure

At one level, this survey documents a familiar set of problems. Parents describe long waitlists, unclear next steps, weak follow-through, inconsistent school communication, gaps in behavioral health support, language barriers, and the ongoing need for advocacy. Parents are left to connect the pieces themselves.

The families participating in this survey describe specific failures in Worcester’s autism support system. Children require continuity, coordination, trust, communication, and durable support across multiple institutions. When those supports are weak, families absorb the strain themselves. The effects of an inadequate social infrastructure become concentrated in the household.

Earlier Worcester parent research on remote learning during COVID-19 identified household strain, communication breakdowns, emotional stress, and inadequate support for students with special needs and IEPs (Worcester Education Justice Alliance, 2020). The present survey documents a distinct but connected set of conditions facing families rearing autistic children. It shows how broader institutional weaknesses interact with the particular need for consistency, behavioral support, language access, specialized education, and long-term coordination. Worcester must build the social infrastructure needed to share the work of rearing children, especially autistic children whose needs require a level of continuity and coordination that fragmented systems cannot provide.

2.3 Scope and Direction of the Report

Families rearing autistic children remain at the center of this report. The analysis begins with the educational, behavioral, clinical, linguistic, and institutional supports their children require. Its broader discussion of Worcester’s social infrastructure grows out of those findings.

The coalition understands this survey as Worcester’s first community-led effort of this scope to document barriers to autism support. Participating families reported problems involving service access, education, behavioral health, language, culture, and advocacy. The report places those experiences within the institutions that shape them and uses the findings to propose changes in service delivery, funding, policy, and public accountability. Parent leadership is central to that process.

3. Methodology

How the survey data were prepared, analyzed, interpreted, and contextualized.

3.1 Survey Development, Distribution, and Data Preparation

The Free Worcester Coalition and its coalition partners, including parent leaders Nelly Medina, Natalie Cintron, and Hope Fisher, organized and distributed the Autism Parent Survey through email, social media, and direct outreach to parents. The survey was open from September 2024 through January 2026 and received partial funding from a grant awarded by the Greater Worcester Community Foundation. Its development took six months, with

questions crafted through consensus during monthly online meetings.

The survey questions were designed to gather the information and data needed by the Free Worcester Coalition and its partners to better understand how to serve parents, caregivers, and service providers facing challenges in connecting with autism-related services and supports. The data collected were intended to inform service providers and educational institutions, influence funding decisions, and promote best practices for supporting Worcester's diverse autism community.

The Autism Parent Survey was made available in English, Spanish, Portuguese, Albanian, Arabic, Vietnamese, Kiswahili, and Haitian Creole in an effort to reach families across Worcester's major language communities. However, only the English- and Spanish-language versions received enough responses to support meaningful analysis. This report is therefore based on a merged review of those two datasets. The survey generated structured responses and open-ended comments. The structured responses allowed the research team to count how often particular barriers and needs were reported. The comments recorded how parents described those experiences in their own words.

Before analysis began, the survey data went through a preparation process to create a usable working file. That process included reviewing the field structure of the original survey exports, separating analytic fields from non-analytic or administrative fields, assigning response IDs, and documenting exclusions. In the English-language dataset, 185 submitted responses were reviewed, and 10 responses were excluded because they appeared non-substantive, synthetic, or otherwise not appropriate to include in a credible findings report. In the Spanish-language dataset, 14 responses were reviewed, and 1 response was excluded because it was out of scope. After that cleaning process, the final merged analysis set included 188 retained responses: 175 from the English-language survey and 13 from the Spanish-language survey.

The survey included a mix of question types, including single-select items, yes/no items, multi-select questions, rating questions, and open-ended responses. Structured questions were summarized using counts and percentages. Because many items were optional, the denominator for each percentage was the number of respondents who answered that question. Multi-select questions were analyzed by counting how many respondents selected each option, which means percentages for those items do not sum to 100 percent.

For multi-select items, semantically equivalent English- and Spanish-language options were harmonized into common categories and counted once per respondent. Some option labels contained commas, so the research team reconstructed the categories from the original response strings and preserved each complete label as one option.

Eight English-language responses and three Spanish-language responses were retained with documented cautions because they remained sufficiently relevant and interpretable for descriptive analysis. A sensitivity check excluding these responses did not materially alter the report's principal findings.

3.2 Qualitative Analysis

Open-ended responses were reviewed thematically. The analysis began with a close reading of the merged open-text responses, including translated Spanish-language responses alongside the original text where relevant. The research team developed themes inductively from the responses themselves. As recurring patterns emerged, responses were grouped into themes such as service navigation, school communication, IEP confusion, behavioral health gaps, language access, caregiver strain, and parent support. Those themes were then reviewed alongside the structured survey results in order to identify the strongest cross-cutting findings in the dataset.

The qualitative coding process was conducted by the primary report author, Addison Turner, with input from members of the Free Worcester team that led the study. When a response was ambiguous or supported more than one reading, the research team interpreted it using the available context and stayed as close as possible to the participant's own language.

For purposes of the qualitative analysis, a coded open-ended response refers to one respondent's answer to one open-ended survey question that was assigned to a thematic category. A single answer could be assigned to more than one theme, and an individual respondent could contribute multiple coded responses to the same theme through answers to different questions. For each theme, the report presents the number of coded open-ended responses and the number of unique respondents who discussed it. These figures should be understood as descriptive measures of how often themes appeared in the qualitative data, not as estimates of their prevalence among all Worcester families.

3.3 Scope and Interpretation

This report does not claim to be a statistically representative study of all families in Worcester. It is better understood as a grounded parent survey that documents recurring experiences, pressures, and priorities among the families who participated. Its strength lies in the combination of breadth and specificity: the structured responses make it possible to identify recurring patterns, while the open-ended responses provide a richer understanding of how those patterns are experienced in everyday life. The analysis examines what the survey results reveal about the conditions under which families are trying to rear autistic children in Worcester.

An exploratory age-based analysis was also conducted. Responses identifying a single age range were grouped into four broader bands: ages 2–6, ages 7–11, ages 12–17,

and ages 19 and older. Five responses identifying children in multiple age ranges were not included in the single-band comparisons. With age information missing from 31 responses and uneven group sizes, especially for children ages 19 and older, the comparisons describe patterns within this survey. They cannot provide population estimates or establish that age caused the reported differences.

3.4 Targeted Narrative Review

To help interpret the survey findings, the research team conducted a targeted narrative review of relevant literature on autism service systems, disability studies, care coordination, family navigation, caregiver support, language access, behavioral health, and parent engagement. Parent testimony remained the primary source of evidence. The literature was used to compare the experiences reported in Worcester with broader research and to inform the report's interpretations and recommendations.

3.5 Local Institutional and Political Context

To situate the survey findings within their local institutional and political setting, the analysis also included a targeted review of Worcester Public Schools program descriptions, School Committee actions, district announcements, local news coverage, and community discussions related to autism services and special education. The local materials documented the formal structure of services, recent policy developments, institutional commitments, and the debates surrounding those developments.

Official district materials were used primarily to establish how programs, services, and decisions have been formally described. News reporting and community discussions were used to document the concerns, questions, and perspectives raised by parents, educators, advocates, and other stakeholders. Reading these sources alongside the survey made it possible to distinguish between the formal existence of programs and families' practical ability to understand, access, navigate, and rely upon them in practice.

3.6 Study Limitations

This grounded qualitative and descriptive survey reflects the experiences of participating families; its sample does not support statistical estimates for all Worcester families or the prevalence of those experiences across the city. The report identifies recurring barriers, pressures, and priorities among the families who chose to participate.

4. Data and Results

What participating families reported about access, schools, behavioral health, language, and advocacy.

Across the merged English- and Spanish-language survey data, four major problem areas emerged clearly: (1) barriers to getting services, (2) school communication and advocacy needs, (3) behavioral health and related support gaps, and (4) language access and cultural responsiveness. A fifth, cross-cutting finding was the substantial interest participants expressed in continued peer connection and advocacy.

4.1 Barriers to Getting Services and Support

One of the clearest patterns in the survey is that many families are left to figure the system out on their own. Even at the front end, parents often described having to piece together information from different places and guess at what the next step was supposed to be. In the structured data, **the most common source of first guidance was self-directed research or online searching (26 respondents, 21.0%),** followed by **school or special education (24 respondents, 19.4%).**

PARENT VOICE

“Transparency / Not told of ALL of the supports available and my rights.”

Among the 178 respondents who answered the question, 69 respondents (38.8%) reported being on a waiting list without adequate follow-up, either because they were not hearing back or because they had not received updates. Insurance was identified as a barrier by 46 respondents (25.8%); 30 respondents (16.9%) said they needed help navigating or understanding information. In the qualitative review, service navigation and unclear next steps appeared in 73 coded open-ended responses contributed by 53 unique respondents. Provider knowledge or autism-specific competence appeared in 99 coded open-ended responses contributed by 66 unique respondents.

SURVEY DATA

38.8% reported being on a waiting list without adequate follow-up.

25.8% identified insurance as a barrier.

17.4% identified transportation as a barrier.

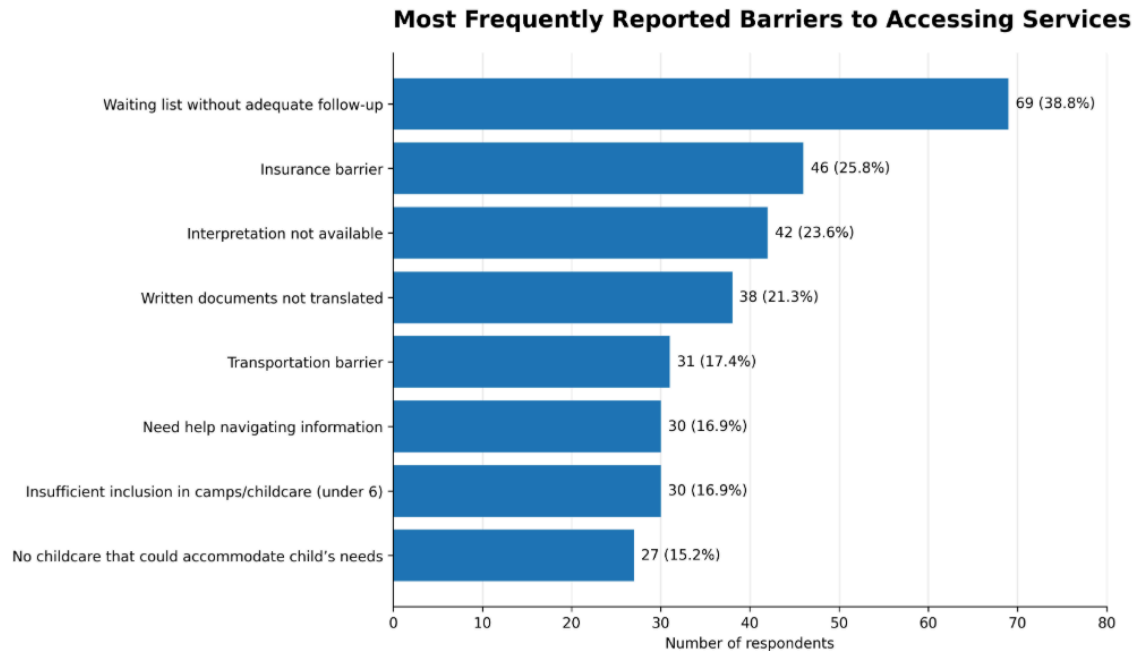


Figure 1. Most Frequently Reported Barriers to Accessing Services. Based on 178 respondents who answered the barriers question. The most commonly reported barrier was being on a waiting list without adequate follow-up, followed by insurance barriers, lack of interpretation, untranslated written information, and practical access barriers such as transportation and childcare.

One parent needed “to figure out where I can test my child for autism.” Another wrote, “Transparency / Not told of ALL of the supports available and my rights.” Transportation was selected as a barrier by 31 respondents (17.4%), insufficient inclusion in camps and childcare settings for children under six by 30 respondents (16.9%), and the inability to find childcare that could accommodate a child’s needs by 27 respondents (15.2%).

KEY FINDING

Families are left to navigate unclear pathways, weak follow-up, and disconnected institutions.

4.2 School Communication, Advocacy, and IEP Navigation

In the structured data, schools, special education, or other educational institutions were collectively the most common source of help, named by 47 respondents (38.5%). The qualitative data show that school communication, IEP follow-through, staffing, and advocacy needs remain among the strongest pressure points in the survey.

PARENT VOICE

"The special education department was supportive, but regular staff do not review the IEP."

An accepted IEP establishes legally enforceable obligations for the school district. Federal regulations require that the IEP be accessible to every teacher and service provider responsible for its implementation and that each person understand their specific responsibilities, including the accommodations, modifications, and supports that must be provided (34 C.F.R. § 300.323(d)). Massachusetts regulations likewise require school districts to implement all accepted elements of an IEP without delay (603 CMR 28.05(7)(b)). When staff do not review or consistently carry out an IEP, the problem is weak communication and is also a matter of legal compliance and the protection of a student's educational rights.

In the coded responses, school communication and special education coordination appeared in 207 coded open-ended responses contributed by 95 unique respondents. IEP process confusion and advocacy needs appeared in 65 coded open-ended responses contributed by 43 unique respondents. School staffing or service-intensity gaps appeared in 77 coded open-ended responses contributed by 27 unique respondents.

Bullying and school safety also emerged as a distinct, though secondary, concern. The broader qualitative category of bullying, school safety, or poor school fit appeared in 35 coded open-ended responses contributed by 32 unique respondents. Because this category also includes unsafe conditions and school-placement concerns, these figures should not be interpreted as 32 reports of bullying alone. Nevertheless, several respondents named bullying directly, including one who said that "bullying was the worst" and another who described their child being bullied on the school bus. For some families, school problems include threats to children's safety, inclusion, and ability to participate without mistreatment or isolation.

In response to the question about school-based opportunities, 50 respondents (29.2%) said they were not aware of them.

SURVEY DATA

38.5% identified schools, special education, or other educational institutions as their most common source of help.

29.2% were unaware of school-based opportunities.

One respondent answered the IEP-related question with just four words: “An advocate to be present.” Another described the contradiction within school support this way: “Special education department was supportive but regular staff do not review the IEP.”

KEY FINDING

Parents often depend on schools for support and encounter some of their strongest difficulties there. Families’ concerns extend from communication and IEP implementation to bullying, safety, belonging, and poor school fit. Formal rights and services matter only when they are consistently implemented and children are safe and included in practice. Because an accepted IEP establishes legally enforceable obligations, inconsistent staff review or implementation is a student-rights and institutional-compliance issue.

4.3 Behavioral Health and Co-Occurring Service Gaps

Among the 181 respondents who answered whether their child also struggled with behavioral health challenges, 143 (79.0%) said yes. Of those 143 respondents, 119 (83.2%) reported having used behavioral health services; 24 (16.8%) had not.

SURVEY DATA

79.0% reported that their child also experienced behavioral health challenges.

Among those families, **83.2%** had used behavioral health services.

“Behavioral health service gaps” was the most frequently assigned theme in the qualitative review, appearing in 234 coded open-ended responses contributed by 78 unique respondents. Missing social, emotional, and life-skills supports appeared in 87 coded open-ended responses contributed by 59 unique respondents. Caregiver health or stress needs appeared in 96 coded open-ended responses contributed by 54 unique respondents.

PARENT VOICE

“I’m happy that I have a therapist for my son, that is one of the very real issues trying to get them and keep them.”

One parent described how fragile access can feel even when support is technically in place: “I’m happy that I have a therapist for my son, that is one of the very real issues trying to get them and keep them.” Another named a more specific gap: “Trying to find a speech therapist that does in-home services for my son after age 3, I am still currently looking.” These responses point to an unmet need for therapy in addition to a broader set of supports around regulation, peer connection, and life skills.

These service gaps also contribute to caregiver strain. Families are carrying school demands, appointments, stress, and service instability at the same time.

4.4 Language Access, Trust, and Cultural Responsiveness

Language access is one of the conditions shaping whether families can use services at all. Among the 178 respondents who answered the barriers question, **42 respondents (23.6%) reported that spoken interpretation services were not available in their language, and 38 respondents (21.3%) reported that written documents were not translated into a language they could use or understand.** Respondents also selected barriers related to autism not being understood or accepted in their culture (32 respondents, 18.0%) and staff not understanding their culture (25 respondents, 14.0%). Language access and cultural responsiveness require separate attention.

Language access and translation appeared in 63 coded open-ended responses contributed by 38 unique respondents. Cultural responsiveness and stigma appeared in 25 coded open-ended responses contributed by 25 unique respondents. One Spanish-language respondent put it plainly: “Many institutions and programs only have information in English. Even therapists often only speak English, and it is difficult to have direct communication.” That is a direct, inequitable barrier to care.

PARENT VOICE

“Many institutions and programs only have information in English. Even therapists often only speak English, and it is difficult to have direct communication.”

(Translated from Spanish)

KEY FINDING

Access is not meaningful unless support is stable, appropriate, culturally responsive, and available in a language families can understand and use.

4.5 Parent Connection, Advocacy, and the Potential for Organization

A major cross-cutting finding is that many participating parents want continued connection, mutual support, and opportunities to advocate beyond the survey itself. Parent support and peer connection appeared in 52 coded open-ended responses contributed by 44 unique respondents; **123 respondents (69.5%) said they would like to become involved in advocacy work.** Parents also directly requested spaces where families could connect, including “opportunities to gather with other families who have kids on spectrum” and “an autism support group for other parents specific to my child’s school.”

PARENT VOICE

“Opportunities to gather with other families who have kids on spectrum.”

Respondents most often identified email, selected by 93 respondents (52.0%), and phone, selected by 58 respondents (32.4%), as the best ways to receive information.

SURVEY DATA

69.5% expressed interest in becoming involved in advocacy.

52.0% preferred receiving information by email.

KEY FINDING

Parent interest creates a real opening for collective action, but lasting organization will require intentional outreach, accessible participation, leadership development, and material support.

4.6 Age and the Changing Form of Family Support Needs

The form of support reported by families varied across childhood, adolescence, and the transition into adulthood. Because the age groups were uneven and the survey was not designed for statistically representative age comparisons, the patterns below are descriptive of this sample.

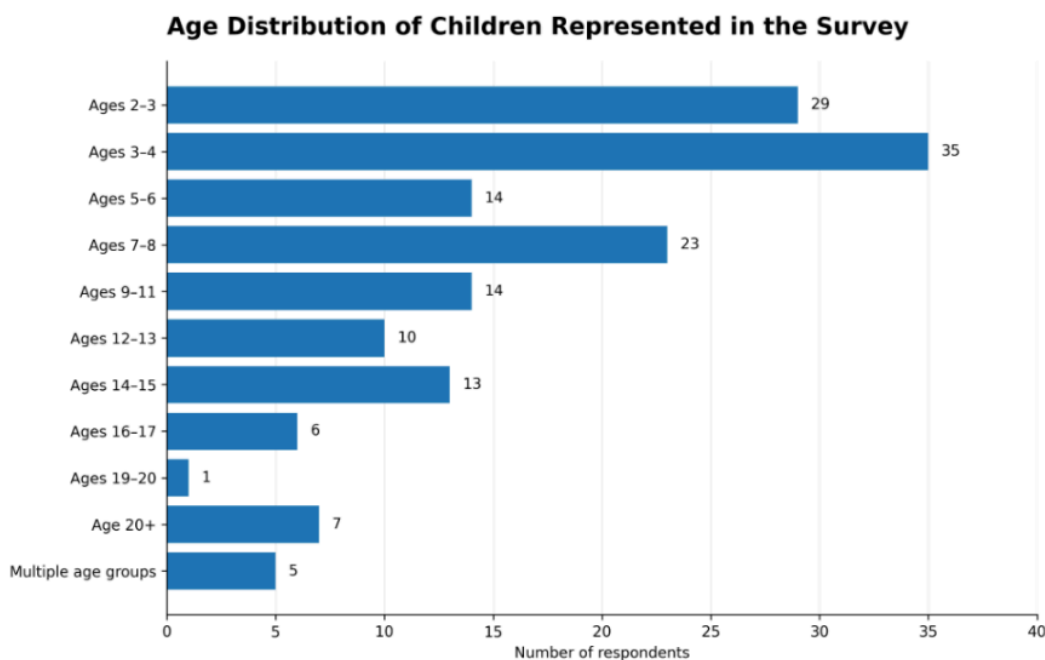


Figure 2. Age Distribution of Children Represented in the Survey. Age information was provided by 157 respondents. Five respondents selected more than one age range. The concentration of responses among families with younger children should be considered when interpreting the report's age-based findings.

Among families with children ages 2–6, the coded responses concentrated heavily on communication and coordination with schools and special education, confusion or need for advocacy around the IEP process, provider knowledge, and the difficulty of identifying an appropriate pathway into services. Childcare and the lack of inclusive camps and care settings also appeared as particularly relevant practical concerns. These responses reflect the demands families encounter while entering early intervention, diagnosis, school-based services, and formal special education systems.

Among families with children ages 7–11, bullying, school safety, and poor school fit became especially visible. The broader qualitative category of bullying, school safety, or poor fit appeared in responses from 17 of the 37 respondents in this age group. The structured school question shows the same concentration, with numerous respondents in the 7–11 group directly selecting bullying as a need faced at their child's school.

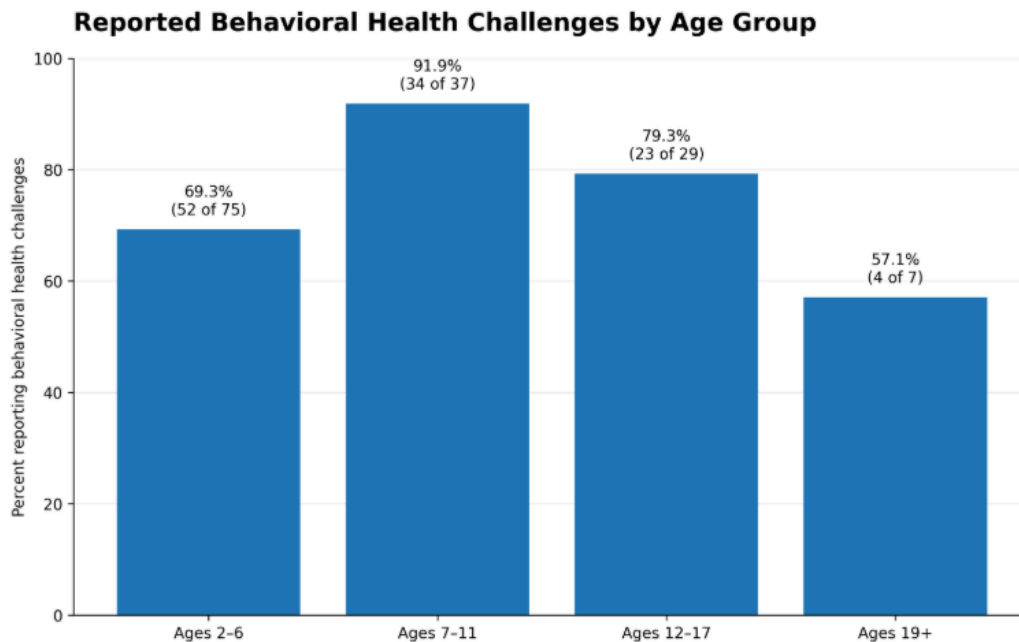


Figure 3. Reported Behavioral Health Challenges by Age Group. Percentages are descriptive of participating families and should not be interpreted as age-specific prevalence estimates for Worcester. Reported behavioral health challenges were especially prominent among families with children ages 7–11 in this survey. The ages 19 and older group is small and should be interpreted cautiously.

Behavioral health challenges were also particularly prominent among families of school-age children. Thirty-four of the 37 respondents with children ages 7–11, or 91.9 percent, reported that their child experienced behavioral health challenges. Among respondents with children ages 12–17, 23 of 29, or 79.3 percent, reported the same, compared with 52 of the 75 respondents with children ages 2–6 who answered the question, or 69.3 percent. These percentages describe the survey sample and are not estimates of age-specific prevalence in Worcester. Behavioral health challenges were reported most frequently among participating families with school-age children.

For families with children ages 12–17, the qualitative data placed greater emphasis on service navigation, behavioral health continuity, caregiver strain, social and emotional development, life skills, and preparation for the transition out of school-based systems. Parents described difficulty finding age-appropriate peer and social opportunities, obtaining practical life experiences, coordinating services, and preparing for housing, employment, further education, and adult support. Families with adolescents continued

to report earlier service and school problems alongside new demands related to transition planning and uncertainty. Examples include requests for practical social and emotional groups, life experiences outside school, coordinated support, and help preparing for what comes after school-based eligibility.

The group representing children and young adults ages 19 and older was too small for meaningful percentage comparison. Still, several of these respondents described a recognizable set of transition-age concerns, including guardianship, SSI and other legal paperwork, higher education, adult services, housing, employment, service eligibility, and the loss of supports after aging out of school. One parent wrote that “everything goes away after the transition age, but these young adults still need the supports,” while others described the legal aspects of transition, the need for information about higher education, and services ending at ages 21 or 22.

The form of support must change as children move from diagnosis and early intervention into school and peer environments, through adolescence, and into adult life. Parents should not have to rebuild their child’s support network at each transition.

5. Local Institutional and Political Context

How Worcester’s institutions, public decisions, and competing priorities shape family experience.

Worcester has a formal network of autism and special education programs, and parents have organized for years around the future of specialized supports. The barriers documented here belong to that larger institutional struggle; treating them as individual confusion or isolated administrative failures would misstate the problem. Formal services have not relieved parents of the need to find their own way through the system, coordinate support across institutions, and live with uncertainty about whether that support will remain in place.

5.1 Formal Services and Lived Access

Worcester Public Schools formally describes a range of specialized services for autistic students and students with related support needs. These include Autism/Life Skills and Specialized Approaches to Individual Learning, commonly known as SAIL, supported by special education teachers, paraeducators, Board Certified Behavior Analysts, and related-service providers. Autism/Life Skills is described as providing individualized instruction, functional skills development, behavioral support, and related services required by a student’s individualized education program (Worcester Public Schools, n.d.; Gates Lane School of International Studies, n.d.).

Participating families described difficulty identifying available services, entering them, and coordinating support across schools, clinical providers, behavioral health services, and community organizations. Their experiences expose the distance between formal availability and a family's ability to rely on support in practice.

An inventory of programs cannot establish that Worcester's system is adequate. The number and size of existing services say little about their quality, accessibility, stability, or whether families can rely on them. Individual training and good intentions do not repair weak coordination between institutions or give families control over the conditions in which their children are raised. Parents are still being forced to act as case managers, navigators, advocates, and coordinators. Families should be able to share that responsibility with institutions they can trust.

5.2 Parent Advocacy and the 2025 Autism Resolution

In April 2025, a parent-led public petition proposed a Worcester School Committee resolution recognizing Autism Acceptance and Awareness Month. The resolution received public support and was passed unanimously by the committee (Doyle, 2025). Autism-related concerns were already the subject of local organizing and public debate before this report was completed.

LOCAL CONTEXT

In April 2025, parent organizing led the Worcester School Committee to unanimously adopt a resolution recognizing Autism Acceptance and Awareness Month.

Parent organizing produced the resolution by directing a concrete demand to the School Committee, the body with authority to adopt it. That public commitment gives families a standard against which to judge later decisions. Its implementation can be assessed through culturally and linguistically accessible information, meaningful professional development for educators, stronger responses to bullying, and more inclusive school practices. Public documentation, measurable benchmarks, and continued review by affected families are necessary to determine whether institutional acknowledgement produces concrete change.

The survey does not establish how prevalent bullying is across Worcester schools, but it confirms that bullying and school safety are concrete concerns for a subset of participating families. The resolution's implementation should therefore be assessed by whether schools document incidents, respond effectively, improve student and staff understanding, and demonstrate that autistic students are safer and more fully included.

Parent organizing around the resolution predates this report, and 123 survey respondents expressed interest in advocacy. The potential base for parent leadership extends beyond those already shaping local discussion.

5.3 The 2026 Restructuring of Specialized Programs

The urgency of these questions increased in May 2026, when Worcester Public Schools announced that the Academic Center for Transition and Worcester Alternative School would close at the end of the school year. Both programs served students with individualized education programs in therapeutic and individualized settings. The district announced that affected students would be referred to programs operated by the Central Massachusetts Collaborative and cited fiscal constraints and the reduction of duplicated services as reasons for the changes (Worcester Public Schools, 2026).

Worcester Public Schools did not describe these schools as autism-specific programs. They are nevertheless relevant to the larger educational environment in which autistic students and their families seek support, particularly because they served students with complex academic, behavioral, and therapeutic needs.

LOCAL CONTEXT

In May 2026, Worcester Public Schools announced the closure of the Academic Center for Transition and Worcester Alternative School, citing fiscal constraints and the reduction of duplicated services.

Parents, educators, and community members publicly challenged the closures. Their concerns included the possible disruption of trusted relationships, established routines, individualized supports, and the stability required by students with significant needs. Participants in the public debate also questioned the transparency of the decision-making process and the extent to which affected families and staff had been meaningfully involved before the changes were announced (Collings, 2026).

The same announcement described the closures as strengthening student outcomes. The district's account joins fiscal necessity and improved support within the same explanation. Families and educators emphasized continuity, trusted relationships, established routines, stability, and individualized knowledge of students. Treating placement as a plug-and-play matter leaves those conditions out. A program that appears equivalent on paper cannot be assumed to function equivalently in a child's life. Families understand what it means to make difficult decisions with limited resources. The district should be equally direct about the tradeoffs it is imposing, explain how it is

weighing competing priorities, and involve affected families before decisions are finalized.

5.4 Implications for the Survey Findings

Parents' difficulties concern the relationships between programs, the stability of those programs, the flow of information between institutions, and the degree of influence families possess over decisions that reshape their children's lives. Program changes can reproduce the same failures they are presented as solving when formal equivalence substitutes for continuity, trusted relationships, and family judgment.

Parents are not meant to rear children alone. Institutions already exercise the authority to define services, set budgets, consolidate programs, and determine which forms of continuity can be sacrificed. Families bear the consequences without comparable power over those decisions. Consultation after a decision has been made leaves that balance intact. Families need authority before decisions are finalized and a means of enforcing public commitments afterward.

Worcester needs stronger connective tissue between schools, providers, public agencies, community organizations, and families. Building those connections requires changes in service delivery and in how institutions share information, make decisions, distribute authority, and remain accountable to the public.

To the extent that some of the local developments described here occurred after the survey was administered, they should not be presented as causes of the responses. They are subsequent developments that heighten the relevance of what parents had already reported and demonstrate how questions of access, continuity, trust, and family participation continue to unfold in practice.

6. Background Research and Systemic Context

How the Worcester findings relate to broader research on disability, caregiving, and fragmented support systems.

6.1 National Research and Fragmented Support Systems

Nationally, autism identification has continued to rise (Shaw et al., 2025). Research also documents persistent disparities in geographic access and service use, including barriers connected to race, ethnicity, and language (Liu et al., 2023; Smith et al., 2020; St. Amant et al., 2018). Other studies identify continuing problems in care coordination, continuity, and the integration of educational, clinical, behavioral health, and community supports (Jafarabadi et al., 2021; Parker et al., 2020; Todorow et al., 2018).

RESEARCH CONTEXT

National research documents persistent disparities in service access, along with continuing problems in care coordination, continuity, and the integration of educational, clinical, behavioral health, and community support.

Within this broader pattern, parents are often required to act as navigators, case managers, advocates, translators, school negotiators, schedulers, therapist finders, and long-term strategists, sometimes all at once. In the Worcester dataset, these demands appear as a transfer of coordination, navigation, and advocacy work from institutions onto private households.

Clearer pathways, better follow-up, stronger school communication, improved language access, and more behavioral health support are necessary responses to immediate administrative failures. The collective structures required to support autistic children and their families remain insufficient.

6.2 Disability, Environment, and Institutional Barriers

The **social model of disability** distinguishes children's embodied, developmental, sensory, communicative, and behavioral support needs from the social and institutional barriers that make those needs harder to meet (Oliver, 1990; Goering, 2015).

Participating families described inaccessible information, fragmented services, inadequate accommodations, weak coordination, and institutions that transfer the work of securing support back onto caregivers. These conditions intensify the difficulties children and families face in meeting real support needs.

6.3 Social Reproduction and Caregiver Burden

The labor required to keep children safe, regulated, educated, connected to care, and moving forward does not disappear when institutions fail. It is transferred into the home as unpaid care, stress, time, vigilance, transportation, advocacy, and constant follow-up. The survey does not independently measure how this work is distributed within participating households. However, research on autism caregiving has found that women, especially mothers, often provide more hours of care and experience greater objective and subjective burden than men. U.S. research also indicates that the employment and earnings consequences associated with raising an autistic child fall primarily on mothers rather than fathers (Cidav et al., 2012; Herrero et al., 2024). The institutional failures reflected in this survey should therefore be situated within the broader gendered organization of the everyday labor required to sustain family life.

6.4 Promising Directions and Comparative Learning

Improvements to information and referral processes would address only part of the problem documented here. Research on family navigation, coordinated and family-centered care, caregiver training embedded in daily life, and integrated service pathways with continuity and accountability offers Worcester several approaches worth studying. Peer support and co-design with families also appear in this literature, though the evidence base for them remains less developed (Attar et al., 2025; Fulceri et al., 2023; Lee et al., 2024; Li et al., 2025; Pacione, 2022; Todorow et al., 2018). These approaches aim to distribute coordination and responsibility more effectively around the child and family.

RESEARCH CONTEXT

Promising approaches include family navigation, coordinated and family-centered care, caregiver training embedded in daily life, integrated service pathways, peer support, and family co-design.

Worcester should systematically study communities, systems, and jurisdictions that have built more coherent approaches to autism support, school coordination, language access, behavioral health integration, and parent leadership. The work would examine what was built, under what conditions, with what results, and which elements could be adapted locally.

6.5 Worcester Context and the Village as Social Infrastructure

An earlier Worcester parent report on remote learning during COVID-19 identified household challenges, communication challenges, mental and emotional well-being concerns, and remote learning challenges (WEJA, 2020). The crisis exposed and intensified weaknesses already present. The present survey extends that local record by documenting how those broader weaknesses take particular forms for families rearing autistic children. These families face particular demands involving education, behavioral health, disability services, and long-term care coordination, with an especially pronounced need for continuity, trust, language access, behavioral support, and specialized services that fragmented systems are poorly equipped to meet.

What is missing is the village that it still takes to raise a child. This should not be understood sentimentally. The “village” means a real social arrangement in which the labor, responsibility, and knowledge required to rear children are shared across durable relationships, institutions, and organized people. Parents are not left to confront schools, providers, and agencies as isolated individuals. Communication is sustained through

relationships, accountability does not depend on which family can push hardest, and the work of rearing children is treated as collective work.

That village does not yet exist at the level participating families need. The task is to develop the organized capacity required to address existing contradictions rather than simply manage them more efficiently.

7. Recommendations and Next Steps

Immediate reforms and longer-term structures for parent organization, accountability, and system building.

Recommendations at a Glance

The report recommends two connected tracks of action: immediate service and access reforms, and long-term organization, accountability, and system building.

Immediate Service and Access Reforms	Long-Term Organization, Accountability, and System Building
Centralized family navigation and waitlist follow-up	An autonomous and materially supported parent organization
Clearer school communication and independent IEP support	A parent-led institutional accountability process
Meaningful interpretation, translation, and cultural responsiveness	Formal commitments, benchmarks, timelines, and public reporting
Improved behavioral health continuity and coordination	Comparative research and locally testable reforms
Transportation, childcare, scheduling, and accessibility support	A phased public action plan extending from the first 90 days through three years

Neither track should be made conditional upon completion of the other. Urgent service improvements should begin immediately; work on parent organization and structural planning should build the capacity to secure, evaluate, and deepen those reforms over time.

7.1 Immediate Service and Access Reforms

1. Establish centralized family navigation and waitlist follow-up

Worcester should establish a clearly identified navigation pathway through which families can learn what services exist, determine eligibility, receive help completing referrals, and obtain regular updates while on waiting lists. Families should not be required to contact multiple institutions repeatedly in order to determine the status of a referral or identify the next step.

The navigation process should include clear points of contact, accessible explanations of available services, assistance resolving insurance or eligibility barriers, and procedures for following up when referrals stall. Families should receive updates even when no placement or service has yet become available.

Navigation should be organized around developmental stages and major transition points. Proactive guidance is especially important when families enter early intervention or school-based services, move between school levels, approach adolescence, prepare for postsecondary education or employment, or near the end of school-based eligibility. Support should begin before an existing service ends or a crisis occurs.

2. Strengthen school communication, independent IEP support, and responses to bullying

Families should receive clear and accessible explanations of IEP rights, available school supports, implementation procedures, and complaint or appeal options. Families who need additional assistance should have access to trained advocates who can help them prepare for and participate in school meetings.

Because an accepted IEP establishes enforceable obligations, the district is responsible for its implementation. Parents should not have to repeatedly identify failures before schools determine whether required services and accommodations are being provided.

Schools should establish procedures for confirming that relevant staff have reviewed each student's IEP and are implementing the required accommodations and services. Communication should not depend entirely on individual parents repeatedly contacting teachers, administrators, and service providers to determine whether agreed-upon supports are being delivered.

Worcester Public Schools must establish clear, accessible procedures to prevent bullying involving autistic students and to report, investigate, and follow up on incidents. Families should know who is responsible for responding, what steps will follow a report, how the student will be protected during the process, and how recurring incidents will be addressed. Prevention should include autism- and neurodiversity-informed education for staff and students, attention to social exclusion and isolation as well as overt harassment, and regular review with affected families.

3. Guarantee meaningful language access

Interpretation and translated materials should be available throughout the service process, beginning at intake. Families should be able to communicate directly with schools, providers, and agencies in a language they understand when discussing eligibility, consent, treatment, IEPs, complaints, service changes, and major decisions affecting their children.

Translation alone cannot resolve barriers created when families believe that providers do not understand their cultural context or when autism is poorly understood within the institutions and communities with which they interact. Language-access standards should be monitored through public reporting and direct feedback from families.

4. Improve behavioral health continuity and coordination

Families need support locating appropriate behavioral health, speech, therapeutic, social-emotional, and life-skills services. That assistance must continue when a provider leaves, a service ends, a child ages out of a program, or the referral process has to begin again.

Schools, clinical providers, behavioral health organizations, and community institutions should develop clearer procedures for coordinating care, sharing appropriate information with family consent, and preventing caregivers from becoming the sole link between disconnected systems. Service contact should not be treated as proof that a family's needs have been adequately met. Continuity, quality, appropriateness, and stability must also be assessed.

5. Address transportation, childcare, and other practical barriers

Transportation, childcare, scheduling, and accessibility are part of service access. A service is not meaningfully accessible when a family cannot reach it, cannot find appropriate care for another child, or cannot attend during the hours offered.

Parent meetings, training, advocacy activities, and major planning processes need transportation assistance, childcare, remote-participation options, interpretation, and, whenever possible, schedules designed around caregiver availability. The longer-term parent organization and accountability process described below needs the same practical supports.

7.2 Long-Term Organization, Accountability, and System Building

1. Build and resource an autonomous parent organization

Free Worcester should begin by inviting survey participants and other affected families into an accessible process of continued communication, mutual support, and

organization. Building durable collective capacity will require intentional outreach, relationship-building, and material support.

Parents must choose their own leaders through an open and accessible process. Reaching families excluded by language, transportation, childcare, work schedules, disability access, or previous negative experiences with institutions demands intentional outreach. Meaningful participation requires interpretation, translated materials, childcare, transportation assistance, remote-participation options, and compensation for substantial leadership or advisory work.

The parent organization should retain the ability to meet independently, develop its own analysis, determine priorities, select representatives, and act publicly in its own name. Free Worcester may initially provide outreach, coordination, meeting space, and administrative support, but it should not permanently control the organization or substitute its own leadership for parent leadership. Institutional partnership matters, but it cannot replace organized parent power.

2. Establish a parent-led accountability and institutional commitment process

A parent-led accountability process must include representatives with actual decision-making authority from Worcester Public Schools, special education leadership, relevant city agencies, behavioral health and clinical providers, language-access offices, and community organizations. Its first task would be to create a shared public record of what the survey establishes, what remains unresolved, and which institutions are responsible for the barriers identified.

The accountability process would require each participating institution to identify the decisions, resources, and policies within its authority and make specific commitments connected to the findings.

Those commitments should be formalized through a memorandum of understanding that names responsible parties, timelines, measurable benchmarks, reporting requirements, and procedures for corrective action when commitments are not met. The MOU should identify what the participating parties agree the findings show, what additional research or documentation is needed, and how progress will be publicly evaluated.

Parents of autistic children should help shape the accountability process, select benchmarks, interpret results, and determine corrective action. Major changes should not be finalized or announced before affected families participate in the decision. Continued public review would cover the 2025 autism resolution, the effects of special education program changes, staffing, program capacity, waitlists, transfers, language access, service continuity, and family experiences.

When fiscal or administrative pressures conflict with continuity, stability, individualized support, or service quality, the city and school district should explain how they weighed

those priorities, whose needs the decision subordinates, and how affected families can intervene before it is finalized.

3. Conduct comparative research and develop locally testable reforms

Worcester should conduct a structured review of how other communities and systems approach family navigation, coordinated care, behavioral health continuity, school-family partnership, language access, caregiver support, peer connection, and parent leadership. The review should document what was built, the conditions and resources behind it, and the results, then identify which elements address Worcester's barriers and can be adapted locally.

Parents should help select, design, and evaluate a limited number of locally testable reforms emerging from the comparative review. Each pilot should identify:

- the barrier it is intended to address;
- the population it is intended to serve;
- the institution responsible for implementation;
- the resources required;
- the duration of the pilot;
- the measures by which success will be evaluated;
- and the process for revising, expanding, or discontinuing it.

Research should lead to action. The completion of another study or planning document should not be treated as an adequate outcome by itself.

4. Adopt a phased public action plan

Worcester should implement these recommendations through a phased public process that names responsible parties, deadlines, resource commitments, and regular opportunities for review.

LAUNCH	BUILD	INSTITUTIONALIZE
First 90 Days	3 to 12 Months	1 to 3 Years
Distribute the findings in accessible languages and formats.	Establish and resource the parent organization.	Evaluate the reforms and pilot programs.
Begin outreach to survey participants and other affected families.	Negotiate a memorandum of understanding among participating institutions.	Expand approaches that demonstrate value.

Identify an interim group of parent leaders.	Select public accountability benchmarks and reporting requirements.	Establish durable funding for successful programs and structures.
Convene institutional representatives with decision-making authority.	Complete the comparative research process.	Integrate parent oversight into major institutional decisions.
Clarify unresolved questions about the data and its implications.	Launch immediate reforms and a limited number of locally testable pilots.	Maintain continuing systems of public reporting and corrective action.

Progress reports should identify what has been completed, what remains delayed, why delays have occurred, how resources have been used, and what corrective action will follow. The long-term objective is to build relationships, institutions, habits, and forms of shared responsibility that do not depend upon a single grant, administration, organization, or staff champion.

8. Conclusion

What the findings demand and the collective capacity Worcester must now build.

Parents who participated in this survey described long waitlists, weak follow-through, school confusion, behavioral health gaps, language barriers, and the continuing burden of coordinating systems that do not reliably coordinate themselves. Families and autistic young people themselves absorb the consequences.

Many participating parents expressed interest in remaining connected, sharing information, and advocating collectively. Improvements in navigation, language access, school support, behavioral health continuity, transportation, and childcare should begin now. Parents need organized power to shape priorities, oversee implementation, and hold institutions accountable over time.

Worcester must build the durable social infrastructure families describe as missing: the village autistic children and their families need to thrive.

References

- Attar, S. M., Benavidez, H., Gicheru, C., Alabi, C., Sapano, R., & Stone, W. L. (2025). Co-designing a novel service delivery pathway to increase access to autism identification and care within a non-profit community organization serving culturally and linguistically diverse families. *Autism*, 29(8), 2097–2110.
<https://doi.org/10.1177/1362361325133570>
- Cidav, Z., Marcus, S. C., & Mandell, D. S. (2012). Implications of childhood autism for parental employment and earnings. *Pediatrics*, 129(4), 617–623.
<https://doi.org/10.1542/peds.2011-2700>
- Collings, J. (2026, May 22). Parents, staff express outrage at Worcester Alternative School closure. *Telegram & Gazette*.
<https://www.aol.com/articles/parents-staff-express-outrage-worcester-011739241.html>
- Doyle, A. (2025, April 26). *WPS in brief April 2025*. Worcester Sucks and I Love It.
<https://www.worcestersucks.email/p/wps-in-brief-april-2025>
- Fulceri, F., Gila, L., Caruso, A., Micai, M., Romano, G., & Scattoni, M. L. (2023). Building bricks of integrated care pathway for autism spectrum disorder: A systematic review. *International Journal of Molecular Sciences*, 24(7), 6222.
<https://doi.org/10.3390/ijms24076222>
- Gates Lane School of International Studies. (n.d.). *About us*. Worcester Public Schools.
<https://www.worcesterschools.org/o/glsosis/page/about-us>
- Goering, S. (2015). Rethinking disability: The social model of disability and chronic disease. *Current Reviews in Musculoskeletal Medicine*, 8(2), 134–138.
<https://doi.org/10.1007/s12178-015-9273-z>

- Herrero, R., Díaz, A., & Zueco, J. (2024). The burden and psychological distress of family caregivers of individuals with autism spectrum disorder: A gender approach. *Journal of Clinical Medicine*, 13(10), 2861.
<https://doi.org/10.3390/jcm13102861>
- Jafarabadi, M. A., Gholipour, K., Shahrokhi, H., Malek, A., Ghiasi, A., Pourasghari, H., & Iezadi, S. (2021). Disparities in the quality of and access to services in children with autism spectrum disorders: A structural equation modeling. *Archives of Public Health*, 79, Article 58. <https://doi.org/10.1186/s13690-021-00577-5>
- Lee, J. D., Terol, A. K., Yoon, C. D., & Meadan, H. (2024). Parent-to-parent support among parents of children with autism: A review of the literature. *Autism*, 28(2), 263–275. <https://doi.org/10.1177/13623613221146444>
- Li, F., Li, Q., Shen, Q., Zhang, X., Leng, H., Liu, Y., & Zheng, X. (2025). Family navigation programs for children with autism spectrum disorder: A scoping review. *Pediatrics*, 155(4), e2024067947.
<https://doi.org/10.1542/peds.2024-067947>
- Liu, B. M., Paskov, K., Kent, J., McNealis, M., Sutaria, S., Dods, O., Harjadi, C., Stockham, N., Ostrovsky, A., & Wall, D. P. (2023). Racial and ethnic disparities in geographic access to autism resources across the US. *JAMA Network Open*, 6(1), e2251182. <https://doi.org/10.1001/jamanetworkopen.2022.51182>
- Massachusetts Department of Elementary and Secondary Education. (n.d.). 603 CMR 28.00: *Special education* (see § 28.05(7)(b)).
<https://www.doe.mass.edu/lawsregs/603cmr28.html?section=all>
- Oliver, M. (1990). *The politics of disablement*. Macmillan.

- Pacione, L. (2022). Telehealth-delivered caregiver training for autism: Recent innovations. *Frontiers in Psychiatry, 13*, 916532.
<https://doi.org/10.3389/fpsy.2022.916532>
- Parker, M. L., Diamond, R. M., & Del Guercio, A. D. (2020). Care coordination of autism spectrum disorder: A solution-focused approach. *Issues in Mental Health Nursing, 41*(2), 138–145. <https://doi.org/10.1080/01612840.2019.1624899>
- Shaw, K. A., Williams, S., Patrick, M. E., Valencia-Prado, M., Durkin, M. S., Howerton, E. M., Ladd-Acosta, C. M., Pas, E. T., Bakian, A. V., Bartholomew, P., Nieves-Muñoz, N., Sidwell, K., Alford, A., Bilder, D. A., DiRienzo, M., Fitzgerald, R. T., Furnier, S. M., Hudson, A. E., Pokoski, O. M., . . . Maenner, M. J. (2025). Prevalence and early identification of autism spectrum disorder among children aged 4 and 8 years—Autism and Developmental Disabilities Monitoring Network, 16 sites, United States, 2022. *MMWR Surveillance Summaries, 74*(2), 1–22.
<https://doi.org/10.15585/mmwr.ss7402a1>
- Smith, K. A., Gehricke, J.-G., Iadarola, S., Wolfe, A., & Kuhlthau, K. A. (2020). Disparities in service use among children with autism: A systematic review. *Pediatrics, 145*(Supplement 1), S35–S46.
<https://doi.org/10.1542/peds.2019-1895G>
- St. Amant, H. G., Schrager, S. M., Peña-Ricardo, C., Williams, M. E., & Vanderbilt, D. L. (2018). Language barriers impact access to services for children with autism spectrum disorders. *Journal of Autism and Developmental Disorders, 48*(2), 333–340. <https://doi.org/10.1007/s10803-017-3330-y>
- Todorow, C., Connell, J., & Turchi, R. M. (2018). The medical home for children with autism spectrum disorder: An essential element whose time has come. *Current Opinion in Pediatrics, 30*(2), 311–317.
<https://doi.org/10.1097/MOP.0000000000000605>

U.S. Department of Education. (n.d.). *34 C.F.R. § 300.323: When IEPs must be in effect.*

Electronic Code of Federal Regulations.

<https://www.ecfr.gov/current/title-34/subtitle-B/chapter-III/part-300/subpart-D/subject-group-ECFR28b07e67452ed7a/section-300.323>

Worcester Education Justice Alliance. (2020, September). *Worcester parent research on remote learning during the COVID-19 pandemic.*

<https://worcesteredjustice.org/wp-content/uploads/2020/11/Worcester-Parent-Research-on-Remote-Learning-During-the-COVID-19-Pandemic-FINAL-1.pdf>

Worcester Public Schools. (n.d.). *Autism / Life Skills.*

<https://www.worcesterschools.org/page/autism-life-skills>

Worcester Public Schools. (2026, May 1). *Changes for ACT, Worcester Alternative, and NCC Secondary Programs in 2026–27.*

<https://www.worcesterschools.org/article/2860194>